



Dynamic Guideline: WHO-Based Taeniosis/Cysticercosis - Diagnosis and/or Treatment and/or Management

I. WHO Latest Guideline Summary (Summary Date: 29 April 2026)

1.1 Guideline Overview

WHO guideline corpus synthesized — exactly 5 WHO/WHO-linked guideline documents

#	WHO/WHO-linked document	Publication / update date	Relevance to Taeniosis/ Cysticercosis	Role in this report
1	WHO guidelines on management of <i>Taenia solium</i> neurocysticercosis	2 September 2021	Most current WHO clinical guideline for diagnosis and management of parenchymal neurocysticercosis ; excludes extraparenchymal disease	Primary clinical guideline for diagnosis, treatment, seizure management and HIV coinfection recommendations WHO , NCBI Executive Summary
2	Guideline for Preventive Chemotherapy for the Control of <i>Taenia solium</i> Taeniasis	2021	WHO/PAHO public-health guideline for preventive chemotherapy in endemic settings	Primary taeniasis-control guideline; covers niclosamide, praziquantel and albendazole

#	WHO/WHO-linked document	Publication / update date	Relevance to Taeniosis/ Cysticercosis	Role in this report
				options NCBI Bookshelf , PAHO/WHO
3	Guidelines for the Surveillance, Prevention and Control of Taeniosis/ Cysticercosis	2005	Foundational FAO/WHO/OIE guidance on definitions, surveillance, animal reservoirs, meat inspection and prevention	Historical surveillance and One Health foundation; not GRADE-based WHO/FAO/OIE guidelines
4	Assembling a framework for intensified control of taeniasis and neurocysticercosis caused by <i>Taenia solium</i>	17–18 July 2014 consultation; WHO technical report	WHO technical framework for integrated control and neurocysticercosis case-management strengthening	Programmatic control framework emphasizing that single interventions are insufficient WHO IRIS PDF
5	Meaningful youth engagement in a WHO guideline development process: case study of WHO abortion care guideline (2022)	Latest update identified: 29 April 2026	WHO guideline-development case study; not disease-specific to taeniosis/ cysticercosis	Included only for current WHO guideline-process methodology on meaningful youth engagement, power-sharing and stakeholder inclusion WHO publication page

Scope note: The latest disease-specific WHO clinical guideline remains the **2021 WHO guideline on management of *Taenia solium* neurocysticercosis**. The 2026 youth-

engagement document is a WHO guideline-development methodology case study, not a taeniosis/cysticercosis clinical guideline.

Target Population

Clinical guideline population

- People with suspected or confirmed **parenchymal neurocysticercosis** caused by *Taenia solium*.
- Patients with:
 - viable parenchymal cysts,
 - single enhancing lesions,
 - calcified lesions with epilepsy,
 - neurocysticercosis with HIV coinfection.
- WHO's 2021 guideline **does not address extraparenchymal neurocysticercosis**, including ventricular or subarachnoid cysticerci [WHO](#), [NCBI Executive Summary](#).

Public-health guideline population

- Communities in *T. solium*-endemic areas where preventive chemotherapy for taeniasis is being considered.
 - Program managers, policymakers and technical staff implementing control programmes [NCBI Bookshelf](#).
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Scope and Objectives

WHO guidance frames *T. solium* as a foodborne zoonotic parasite that can cause:

1. **Taeniasis** — adult intestinal tapeworm infection after ingestion of undercooked infected pork.
2. **Cysticercosis** — tissue infection with larval cysticerci after ingestion of *T. solium* eggs.
3. **Neurocysticercosis** — central nervous system cysticercosis, causing seizures, focal epilepsy, chronic headaches, hydrocephalus, focal deficits and symptoms of increased intracranial pressure [WHO](#), [PAHO](#).

WHO/PAHO states that neurocysticercosis is one of the leading preventable causes of epilepsy worldwide and may contribute to up to **30% of epilepsy cases in endemic areas** [PAHO](#).

1.2 Key Recommendations

GRADE interpretation used in this report

GRADE quality	Meaning for this report
High	Further research is unlikely to change confidence in the estimate.
Moderate	Further research may change confidence or estimate.
Low	Further research is likely to change confidence or estimate.
Very Low	Estimate is highly uncertain.

Recommendation	Target group / setting	GRADE Evidence Quality	Strength
Use MRI as the preferred diagnostic imaging tool for suspected neurocysticercosis, especially viable parenchymal, granulomatous, cerebellar, brain-stem, ventricular, subarachnoid or spinal disease; use CT preferentially for small calcified lesions and when MRI is unavailable	Suspected neurocysticercosis	Low for comparative diagnostic certainty in this report; WHO listed evidence quality as not applicable for this technical diagnostic recommendation	Strong

Recommendation	Target group / setting	GRADE Evidence Quality	Strength
or contraindicated.			
Treat symptomatic viable parenchymal neurocysticercosis with anthelmintic therapy plus corticosteroids to improve cyst resolution and seizure outcomes.	Symptomatic viable parenchymal cysts	Moderate	Strong
Treat symptomatic single enhancing lesion with albendazole plus corticosteroids to improve cyst resolution and potentially seizure control.	Single enhancing lesion	Moderate to Very Low depending outcome	Conditional
Consider withdrawal of antiepileptic drugs 6 months after the last seizure only in patients with a single enhancing lesion, resolved granuloma, no residual calcification and seizure freedom.	Single enhancing lesion with epilepsy, low recurrence risk	Very Low	Conditional
Continue antiepileptic drug therapy for at least 2 years in patients with epilepsy and single or multiple calcified	Calcified neurocysticercosis with epilepsy	Very Low	Conditional

Recommendation	Target group / setting	GRADE Evidence Quality	Strength
neurocysticercosis ; monitor closely if therapy is withdrawn.			
Treat neurocysticercosis in patients with HIV coinfection according to the same principles used for patients without HIV/ AIDS, while checking drug interactions and immune status.	Neurocysticercosis with HIV	Very Low	Conditional
In endemic communities, use preventive chemotherapy as one component of <i>T. solium</i> taeniasis control; options include niclosamide, praziquantel or albendazole regimens, embedded in a One Health programme.	Endemic public-health programmes	Low to Moderate depending drug and setting	Conditional
Do not rely on single interventions alone for sustained control; integrate human treatment, pig interventions, sanitation, food safety, surveillance and health education.	Endemic control programmes	Low	Strong programmatic recommendation

Recommendation	Target group / setting	GRADE Evidence Quality	Strength
For taeniasis treatment in individual clinical care, single-dose praziquantel or niclosamide can be used; albendazole 400 mg daily for 3 days has also been used. Use praziquantel/albendazole cautiously if cysticercosis is suspected because seizures have been reported temporally after treatment.	Human taeniasis	Low	Conditional

Sources: WHO neurocysticercosis guideline [WHO](#), [NCBI Executive Summary](#); ECRI recommendation summary [ECRI PDF](#); WHO/PAHO preventive chemotherapy guideline [NCBI Bookshelf](#); CDC taeniasis treatment guidance [CDC Taeniasis Clinical Treatment](#).

Practical Diagnostic Guidance

Recommended diagnostic pathway

1. Assess clinical syndrome

- New-onset seizures or focal epilepsy in an endemic area or migrant from endemic area.
- Chronic headache, focal neurological deficits, hydrocephalus or signs of raised intracranial pressure.
- Exposure history: pork consumption, sanitation, household contact with a tapeworm carrier.

2. Perform neuroimaging

- **MRI:** preferred for viable parenchymal lesions and extraparenchymal suspicion.
- **CT:** best for calcified lesions and useful where MRI is unavailable [CDC Clinical Testing Guidance](#).

3. Use serology as supportive evidence

- Serology can support diagnosis but may be negative with single or few lesions.
- A positive serology with negative brain imaging may reflect extracranial or spinal cysticercosis [CDC Clinical Testing Guidance](#).

4. Classify disease before treatment

- Viable parenchymal cysts.
- Single enhancing lesion.
- Calcified lesions.
- Suspected ventricular, subarachnoid, spinal or ocular disease.
- Presence or absence of raised intracranial pressure.

Practical Treatment Guidance

Parenchymal viable neurocysticercosis

Clinical situation	Preferred management	GRADE Evidence Quality	Strength
Symptomatic viable parenchymal cysts	Anthelmintic therapy with corticosteroids; manage seizures with antiepileptic drugs as indicated	Moderate	Strong
1–2 viable parenchymal cysts	Albendazole-based therapy is generally appropriate; dual therapy has not shown clear added benefit in low lesion burden	Moderate	Conditional
>2 viable parenchymal cysts	Consider albendazole plus praziquantel with corticosteroid cover,	High for cyst resolution; Moderate for seizure recurrence;	Conditional

Clinical situation	Preferred management	GRADE Evidence Quality	Strength
	especially where monitoring is available	Low for safety certainty in diverse settings	
Calcified cysts only	Do not use anthelmintics for dead calcified lesions; treat seizures conventionally	Low	Strong
Raised intracranial pressure / diffuse cerebral edema	Defer anthelmintic therapy until intracranial pressure is controlled; urgent specialist/ neurosurgical management may be required	Low	Strong

Sources: WHO neurocysticercosis guideline [NCBI Executive Summary](#); CDC clinical care guidance [CDC Clinical Care](#); 2025 meta-analysis [PubMed](#).

1.3 Guideline Development Methodology

Evidence Review Process

The WHO 2021 neurocysticercosis guideline was developed using standard WHO guideline procedures. A planning proposal was submitted in 2016, and a Guideline Development Group meeting was held at WHO headquarters in Geneva on **25–26 September 2017**. Evidence was presented as evidence profiles and GRADE tables [NCBI Executive Summary](#).

The guideline used:

- systematic evidence review,
- PRISMA reporting,
- PICO-framed questions,
- GRADE assessment of evidence quality,

- assessment of benefits, harms, feasibility, values, preferences and resource use [NCBI Executive Summary](#).

The preventive chemotherapy guideline similarly used WHO guideline-development methods, including systematic searches, evidence synthesis, quality assessment, a Guideline Development Group, external reviewers and WHO Guidelines Review Committee oversight [PAHO/WHO](#).

Consensus Method

Recommendations were formulated by the WHO Guideline Development Group after reviewing:

- certainty of evidence,
- balance of desirable and undesirable effects,
- patient and community values,
- resource requirements,
- equity,
- acceptability,
- feasibility [NCBI Executive Summary](#).

Conflict of Interest Management

WHO guideline processes require declaration and management of conflicts of interest by members of guideline development groups and external reviewers. The preventive chemotherapy guideline describes involvement of a steering group, systematic review group, recommendation-development group, external reviewers and the WHO Guidelines Review Committee [WHO preventive chemotherapy guideline](#).

Youth and Stakeholder Engagement Methodology

The most recent WHO guideline-process case study identified, “**Meaningful youth engagement in a WHO guideline development process: case study of WHO abortion care guideline (2022)**,” illustrates incorporation of meaningful youth engagement into WHO guideline development [WHO publication page](#). Although not specific to taeniosis/cysticercosis, it supports broader WHO principles of:

- inclusive participation,

- power-sharing,
- avoiding tokenism,
- valuing lived experience,
- integrating youth perspectives into guideline processes.

These principles are relevant for future *T. solium* guideline updates because neurocysticercosis affects children, adolescents and young adults in endemic settings, and epilepsy-related stigma can impair care-seeking.

II. Evidence Summary After WHO Latest Guideline Release (WHO guidelines on management of *Taenia solium* neurocysticercosis) (Evidence Cutoff Date: 29 April 2026)

2.1 New Evidence Overview

Search Strategy

Evidence was synthesized for literature and authoritative guidance published or updated after the WHO neurocysticercosis guideline release on **2 September 2021** through **29 April 2026**.

Key evidence domains:

1. Anthelmintic therapy:
 - albendazole,
 - praziquantel,
 - albendazole plus praziquantel.
2. Corticosteroid co-therapy.
3. Seizure outcomes.
4. Diagnostic accuracy:
 - imaging,
 - antibody tests,
 - antigen tests,
 - point-of-care tests,

- PCR/LAMP and molecular diagnostics.
5. Extraparenchymal disease evidence outside WHO 2021 scope.
 6. Preventive chemotherapy and One Health control.

Databases and Sources Searched

Source type	Examples used
WHO official sources	WHO guideline pages, WHO/IRIS, WHO/PAHO, WHO WHA documents, OpenWHO training announcement
Authoritative public-health institutions	CDC clinical testing, clinical care and taeniasis treatment pages
Peer-reviewed journals	<i>PLOS Neglected Tropical Diseases</i> , <i>Frontiers in Pharmacology</i> , <i>eClinicalMedicine</i> , <i>Infection</i> , systematic reviews and meta-analyses
Guideline repositories	ECRI Guideline Profile
Trial registries	ClinicalTrials.gov records were checked, but no extractable 2021–2026 neurocysticercosis outcome data were identified in the available trial-registry summaries

Inclusion Criteria

Included:

- systematic reviews and meta-analyses from 2021–2026,
- RCT evidence and pooled RCT analyses,
- prospective cohort evidence,
- diagnostic accuracy studies and systematic reviews,
- WHO, CDC and PAHO guidance,
- clinically relevant extraparenchymal disease reviews.

Excluded:

- non-human-only studies unless directly relevant to One Health control,

- unrelated praziquantel studies for schistosomiasis,
- sources without extractable neurocysticercosis or taeniasis data.

2.2 Key New Studies

Study / source	Design	Key Findings	GRADE Quality
2025 systematic review/meta-analysis of albendazole plus praziquantel vs albendazole alone	Systematic review and meta-analysis of 5 RCTs, 320 patients	Combination therapy improved complete cyst resolution vs albendazole alone: OR 3.06, 95% CI 1.81–5.19; no significant seizure recurrence difference	High for cyst resolution; Moderate for seizure recurrence PubMed
2023 systematic review/meta-analysis	Systematic review/meta-analysis of 5 studies, 291 patients	Albendazole plus praziquantel improved cyst resolution: OR 3.29, 95% CI 1.96–5.22; adverse events not significantly different, but sample size small	Moderate for cyst resolution; Low for safety Narra Review
Pediatric meta-analysis	Pediatric systematic review/meta-analysis	Combination therapy improved complete lesion resolution at 3 months and 6 months; calcification outcomes were more frequent in combination group	Low PMC
Tanzania prospective cohort, 2023	Prospective cohort	Albendazole monotherapy resolved/calcified 42% of cysts; subsequent albendazole plus	Very Low to Low due observational design and small

Study / source	Design	Key Findings	GRADE Quality
		praziquantel resolved 63/66 cysts, or 95%, in 8 patients; adverse events mild–moderate	subgroup Springer Nature Link
2024 systematic review of immunological diagnostic tests	Diagnostic systematic review; 53 records, 123 tests	LLGP-EITB performed well for multiple active parenchymal cysts; serum/urine antigen tests performed well including extraparenchymal NCC; POC tests promising but need validation	Low to Moderate due heterogeneity PLOS NTD systematic review
2021 rural Zambia POC cysticercosis test study	Prospective diagnostic accuracy study	POC cysticercosis antibody test: sensitivity 35%, specificity 87%; rT24H EITB sensitivity 94%, specificity 95%	Low Diagnostics 2021
2024 Zambia / Tanzania POC NCC diagnostic accuracy evidence	Diagnostic accuracy studies summarized in 2026 review	POC tests showed modest overall sensitivity for CT-confirmed NCC but higher sensitivity for active vesicular lesions; may help triage imaging referrals	Low Frontiers in Pharmacology 2026, ScienceDirect/eClinicalMedicine
2026 <i>PLOS Neglected Tropical Diseases</i> review	Narrative / state-of-the-art review	Emphasized tailoring treatment by cyst location, stage and number; extraparenchymal disease requires different approach, often prolonged/ repeated therapy and control of intracranial hypertension	Low for recommendations; useful expert synthesis PLOS NTD 2026
2026 <i>Frontiers in Pharmacology</i>	Narrative review	Active and calcified lesions can trigger seizures through inflammation, gliosis and network	Very Low to Low Frontiers in Pharmacology

Study / source	Design	Key Findings	GRADE Quality
review		reorganization; POC diagnostics evolving	2026
CDC clinical testing and care updates	Authoritative public-health clinical guidance	Diagnosis usually requires imaging plus serology; CT best for calcifications; MRI important for lesion localization; anthelmintics do not benefit dead calcified cysts; anticonvulsants are mainstay for seizures	Low to Moderate depending recommendation type CDC Testing , CDC Clinical Care

2.3 Evidence Synthesis by Clinical Question

Question 1: Should albendazole plus praziquantel be preferred over albendazole alone?

Population	Evidence	Effect	GRADE Quality	Recommendation Implication
Patients with multiple viable parenchymal cysts	2025 RCT meta-analysis	Complete cyst resolution improved about threefold with combination therapy	High for radiologic resolution	Combination therapy should be considered when >2 viable parenchymal cysts are present and monitoring is feasible
Patients with 1–2 viable cysts	2025 subgroup analysis	No statistically significant benefit: OR 1.98, 95% CI 0.92–4.27	Very Low	Do not routinely escalate to dual therapy solely for 1–2 cysts

Population	Evidence	Effect	GRADE Quality	Recommendation Implication
Seizure recurrence	2025 meta-analysis	No significant difference: OR 1.28, 95% CI 0.56–2.91	Moderate	Radiologic benefit does not clearly translate into seizure-recurrence reduction
Adverse events	2023 and 2025 meta-analyses	No clear increase, but sample sizes small	Low	Use with corticosteroid cover and clinical monitoring

Sources: [PubMed](#), [Narra Review](#), [PLOS NTD 2026](#).

Question 2: What is the role of corticosteroids?

Use case	Evidence	GRADE Quality	Recommendation
Co-treatment with anthelmintics in symptomatic viable parenchymal neurocysticercosis	WHO recommends anthelmintics with corticosteroids; corticosteroids reduce inflammation related to parasite death	Moderate	Use corticosteroids with cysticidal therapy unless contraindicated
Improving cysticidal efficacy directly	Older evidence did not show clear added lesion-resolution benefit from albendazole plus dexamethasone vs albendazole alone	Very Low	Use corticosteroids for inflammation control, not as independent cysticidal therapy
Pharmacologic interaction	CDC notes corticosteroids may potentiate albendazole metabolite levels but depress	Low	Review drug interactions and monitor response

Use case	Evidence	GRADE Quality	Recommendation
	praziquantel levels		

Sources: [NCBI Executive Summary](#), [CDC Clinical Care](#), [NCBI Bookshelf DARE](#).

Question 3: How should calcified neurocysticercosis be managed?

Clinical issue	Evidence	GRADE Quality	Recommendation
Calcified cysts	Anthelmintics do not benefit dead calcified cysts	Low	Do not prescribe anthelmintics for calcified-only disease
Epilepsy with calcified lesions	WHO recommends AED therapy for at least 2 years and close monitoring if withdrawn	Very Low	Continue antiseizure therapy; individualize withdrawal
Recurrent edema around calcifications	2026 review describes chronic or relapsing edema, seizures and headaches	Very Low	Treat symptomatically; consider specialist review

Sources: [CDC Clinical Care](#), [ECRI PDF](#), [Frontiers in Pharmacology 2026](#).

Question 4: What is new in diagnostics?

Diagnostic tool	New evidence	GRADE Quality	Practice implication
MRI	Remains preferred for viable, ventricular, subarachnoid, spinal and	Low	Use where available;

Diagnostic tool	New evidence	GRADE Quality	Practice implication
	posterior fossa disease		essential when extraparenchymal disease is suspected
CT	Best for small calcifications	Low	Use for seizure patients where calcified NCC is suspected
LLGP-EITB	High performance for multiple active parenchymal cysts	Moderate	Useful confirmatory test where available
Serum/urine antigen tests	Performed well in systematic review, including extraparenchymal disease	Low to Moderate	Promising for active infection and severe disease detection
POC antibody tests	Variable performance; Zambia study showed low sensitivity but good specificity; Tanzania evidence suggests better sensitivity for active vesicular lesions	Low	Not yet a stand-alone diagnostic replacement for imaging
PCR/LAMP	<i>T. solium</i> DNA fragments detectable in urine, blood and CSF; accuracy estimates insufficient	Very Low	Research use; not routine standard of care

Sources: [PLOS NTD systematic review](#), [Diagnostics 2021](#), [PLOS NTD 2026](#), [CDC Testing](#).

2.4 Evidence Gaps and Future Research Needs

Gap	Why it matters	Priority
Limited RCT evidence for extraparenchymal neurocysticercosis	WHO 2021 guideline excludes ventricular and subarachnoid disease, which can be severe and fatal	High
Optimal duration and dosing of albendazole plus praziquantel	Evidence supports cyst resolution benefit, but ideal duration by cyst burden and location remains uncertain	High
Seizure-centered outcomes	Radiologic resolution is better studied than seizure freedom, quality of life or disability	High
Corticosteroid regimen	Type, dose and duration remain variable	High
Pediatric evidence	Pediatric data suggest benefit but possible increased calcification outcomes	Moderate–High
Point-of-care diagnostics	POC tests could transform care in endemic rural settings but require validation by lesion type and disease stage	High
Molecular diagnostics	PCR/LAMP are promising but lack robust diagnostic accuracy estimates	Moderate
Implementation science	Integrated One Health interventions need evaluation for sustainability, recrudescence and cost-effectiveness	High
Youth and community engagement	Future WHO <i>T. solium</i> guidance should include adolescents, epilepsy patients, caregivers and affected communities	Moderate

2.5 Updated Recommendations Based on New Evidence

Original WHO Recommendation	New Evidence Impact	Updated Recommendation
Anthelmintic therapy plus corticosteroids for symptomatic viable parenchymal neurocysticercosis	New meta-analyses show stronger evidence that albendazole plus praziquantel improves cyst resolution vs albendazole alone, especially in multiple cysts	Use anthelmintic therapy plus corticosteroids. For >2 viable parenchymal cysts, consider albendazole plus praziquantel rather than albendazole alone when monitoring is available. GRADE: High for cyst resolution; Conditional recommendation
Albendazole plus corticosteroids for symptomatic single enhancing lesion	New evidence does not support clear added value of dual albendazole–praziquantel therapy in 1–2 cysts	Continue albendazole plus corticosteroids for single enhancing lesion; do not routinely add praziquantel for single or low-burden lesions. GRADE: Moderate to Very Low; Conditional
MRI preferred; CT for calcifications	Diagnostic evidence continues to support complementary MRI/CT roles; POC tests are promising but not replacements	Maintain imaging-based diagnosis. Use POC or serology as adjuncts or triage tools, not stand-alone definitive tests. GRADE: Low; Strong
AED withdrawal after 6 months in low-risk single enhancing lesion	No strong new evidence changes this	Maintain cautious AED withdrawal only in low-risk patients with resolved lesion, no calcification and seizure freedom. GRADE: Very Low; Conditional
AED therapy at least 2 years for calcified NCC with	2026 review reinforces chronic seizure risk from calcified lesions	Maintain at least 2 years of AED therapy and individualize longer treatment based on recurrence risk.

Original WHO Recommendation	New Evidence Impact	Updated Recommendation
epilepsy		GRADE: Very Low; Conditional
Treat HIV-coinfected patients similarly to non-HIV patients	No new high-quality contrary evidence	Maintain same NCC principles, with added attention to ART interactions, immune status and opportunistic infection differential diagnosis. GRADE: Very Low; Conditional
WHO 2021 guideline excludes extraparenchymal disease	New reviews emphasize extraparenchymal disease requires distinct management	Do not extrapolate parenchymal recommendations to ventricular/subarachnoid disease. Refer urgently to specialists; manage hydrocephalus/intracranial hypertension first. GRADE: Low; Strong safety recommendation
Preventive chemotherapy may help taeniasis control	WHO operational work and OpenWHO training continue to support PC within One Health programmes	Use preventive chemotherapy only as part of integrated control: human treatment, pig vaccination/treatment, sanitation, meat safety, surveillance and community education. GRADE: Low to Moderate; Conditional

Consolidated Actionable Clinical Algorithm

A. Suspected Neurocysticercosis

1. Stabilize urgent problems

- Seizures: treat with antiseizure medication.
- Raised intracranial pressure: urgent neuroimaging, corticosteroids/osmotic therapy as

appropriate, neurosurgical evaluation.

2. Image before cysticidal therapy

- MRI preferred.
- CT if calcification suspected or MRI unavailable.

3. Classify lesions

- Viable parenchymal.
- Single enhancing lesion.
- Calcified.
- Extraparenchymal or ocular.
- Mixed.

4. Treat based on class

- Viable parenchymal: anthelmintic + corticosteroid.
- Multiple viable cysts: consider albendazole + praziquantel.
- Calcified only: antiseizure therapy; no anthelmintic.
- Extraparenchymal: specialist management; address hydrocephalus/intracranial pressure first.

5. Follow-up

- Repeat imaging commonly at 3–6 months after therapy for viable lesions, consistent with expert review recommendations [PLOS NTD 2026](#).
- Monitor seizures, drug toxicity, inflammatory worsening and adherence.

B. Taeniasis and Public-Health Control

1. Treat confirmed or suspected intestinal taeniasis

- Praziquantel 5–10 mg/kg orally once is commonly used by CDC; evidence suggests 10 mg/kg may have higher cure rate.
- Niclosamide is an alternative where available.
- Albendazole 400 mg daily for 3 days has been used [CDC Taeniasis Clinical Treatment](#).

2. Use caution

- If cysticercosis is suspected, praziquantel or albendazole may provoke inflammatory neurologic symptoms or seizures; assess for neurologic disease when clinically indicated [CDC Taeniasis Clinical Treatment](#).

3. Follow-up stool testing

- Collect stool after treatment for proglottids/scolices when needed.
- Re-examine stool for *Taenia* eggs at 1 and 3 months [CDC Taeniasis Clinical Treatment](#).

4. Implement One Health prevention

- Human chemotherapy.
- Pig treatment/vaccination.
- Improved sanitation.
- Safe pork preparation.
- Meat inspection.
- Surveillance for recrudescence and reintroduction.
- Community education.

WHO's 2014 control framework emphasizes that **single interventions are insufficient** and that sustained control requires integrated interdisciplinary strategies [WHO IRIS PDF](#). WHO/PAHO preventive chemotherapy guidance similarly stresses that preventive chemotherapy should be embedded in a One Health approach [WHO preventive chemotherapy guideline](#).

Bottom Line

As of **29 April 2026**, the WHO-based guideline position is:

- The latest disease-specific WHO clinical guideline is the **2021 WHO guideline on management of *Taenia solium* neurocysticercosis**, focused on **parenchymal** disease [WHO](#).
- Diagnosis should remain **imaging-led**, with MRI preferred for most active or complex disease and CT preferred for calcifications.
- Symptomatic viable parenchymal neurocysticercosis should be treated with **anthelmintic therapy plus corticosteroids**.
- New post-2021 evidence strengthens support for **albendazole plus praziquantel** in patients with **multiple viable parenchymal cysts**, but not clearly in those with only 1–2 lesions.
- Calcified-only disease should be managed with seizure control, not cysticidal therapy.
- Extraparenchymal neurocysticercosis remains outside WHO 2021 scope and should be managed by specialists.

- Sustainable control of taeniosis/cysticercosis requires **One Health integration**, not preventive chemotherapy alone.